
SPOLICY

All Programs

When an individual/client, household, or provider receives benefits exceeding what they were eligible to receive, the Michigan Department of Health and Human Services (MDHHS) must attempt to recover the overpayment (OP).

This item explains overpayment types, responsibilities, and timeframes.

DEFINITIONS

Benefit Recovery System (BRS)

The system within the Bridges application that contains all established Family Independence Program (FIP), State Disability Assistance (SDA), Refugee Cash Assistance (RCA), Child Development and Care (CDC) and Food Assistance Program (FAP) overpayment claims.

Benefit Withholding (BW)

A process where some or all the benefits a recipient is otherwise eligible to receive are withheld and applied as a payment to an overpayment claim owed to the State of Michigan. Also known as **Administrative Recoupment (AR)**.

Discovery Date

The date when an overpayment claim is known to exist, and the amount of the overpayment claim can be reasonably determined.

For IPV overpayment claims the discovery date is determined by the individual's signed and dated disqualification agreement, an IPV hearing request, or prosecutorial referral.

Establishment Date

The date an overpayment claim is created in Bridges and cannot be prior to the discovery date.

Overpayment

The amount of benefits issued to the individual/client, household, or provider exceeding what the recipient(s) was/were eligible to

receive. An overpayment is also the amount of benefits misused, trafficked, or attempted to be trafficked.

Recoupment

The identification, establishment, and recovery of benefit overpayments.

OVERPAYMENT TYPES

All Programs

Types of overpayments include Agency Error (AE), Client Error (CE), Intentional Program Violation (IPV), and Provider Error.

Agency Error

A type of overpayment or underissuance resulting from an incorrect action or failure to take action by the state agency; see [Bridges Administrative Manual \(BAM\) 705](#).

Client Error

A type of overpayment or underissuance resulting from inaccurate reporting on the part of the household; see [BAM 715](#).

Client Suspected IPV

A type of overpayment resulting from suspected intentional inaccurate reporting on the part of the household. FIS/ES will refer **client errors** suspected of IPV to the Office of Inspector General (OIG).

Intentional Program Violation

IPV ([BAM 720](#)) consists of having intentionally:

- Made a false or misleading statement, or misrepresented, concealed, or withheld facts; **or**
- Committed any act that constitutes a violation of FAP, Supplemental Nutrition Assistance Program (SNAP) regulations, or any federal or state law, involving the use, presentation, transfer, acquisition, receipt, possession, theft, trafficking or sale of items purchased with FAP benefits or Electronic Benefits Transfer (EBT) cards.

Provider Error

A type of overpayment claim resulting from a client receiving more benefits than they were eligible to receive because the client/CDC provider gave incorrect or incomplete information or failed to notify the department of required information, believed to be unintentional; see [BAM 715](#).

CDC Provider Suspected IPV

Suspected provider IPV is an overpayment caused by a provider's intentional act to receive higher reimbursements than they were entitled to. See [Bridges Eligibility Manual \(BEM\) 707](#) for information on provider IPVs. If care was authorized, provided, and paid for but the client was ineligible, this would be considered a CDC client error or agency error.

Multiple Overpayment Types

An overpayment may involve more than one overpayment type. If an agency error and client error occur in the same OP period, process as an agency error.

If an overpayment involves suspected IPV along with another overpayment type, an overpayment referral should be routed to OIG by completing an Overpayment Referral in Bridges and ensuring Yes is checked for Suspected IPV.

Prudent judgement should be used in evaluating an overpayment for suspected IPV; see [BAM 720](#) for IPV referral criteria.

CDC Only

If agency error and another error occurred, calculate which error resulted in the largest overpayment and establish the overpayment claim using that error type.

If provider error and CDC individual/client error or CDC individual/client IPV occurred and care was authorized by MDHHS and paid by the Michigan Department of Lifelong Education, Advancement, and Potential (MiLEAP) to the provider, but care was not provided, recover from the provider.

MINIMIZATION OF OVERPAYMENTS

All Programs

MDHHS must minimize potential overpayments by following [BAM 105](#) requirements and by informing the individual/client or authorized representative (AR) of the following:

- Applicants and recipients are required by law to give complete and accurate information about their circumstances. Incorrect, late reported, or omitted information can result in benefit repayment or reduction.
- Applicants and recipients are required by law to promptly notify MDHHS of all changes in circumstances within 10 days (or other timeframe specified in policy) that potentially affects eligibility or benefit amount.
- Applicants can file a timely hearing request, because of a negative action, to suspend a proposed benefit reduction and have their former level benefits reinstated. Reinstated benefits may be recovered in certain instances; see [BAM 600](#).

MDHHS will minimize potential overpayments by referring questionable information to the OIG Front End Eligibility (FEE) agent for investigation via link: [FEE Referral Form](#).

DISCOVERY OF POTENTIAL OVERPAYMENTS

All Programs

A potential overpayment might be discovered through normal casework or by one of the following but not limited to:

- Case readings.
- Computer cross matches.
- FAP Quality Control (QC) audit findings.
- Federal CDC audit findings.
- Welfare Fraud referrals.
- Non-honored repay agreements.
- Michigan Office of Administrative Hearings and Rules (MOAHR).

FAP Only

If discovered that a FAP household is receiving FAP **and** Indian Tribal Food Distribution benefits in the same month, a FAP OP exists **only** if the household received Indian Tribal Food Distribution benefits prior to its FAP application; see [BEM 222](#).

CDC Only

An individual/client or CDC provider may voluntarily repay any program benefits even when there is no overpayment. Refer these situations to the CDC office within MiLEAP: 1-866-990-3227.

**OVERPAYMENT
THRESHOLD****FIP, SDA, RCA, CDC and FAP**

Agency and client errors are not pursued if the OP amount is equal to or less than \$250 per program.

Threshold limits:

- Above \$250 effective December 1, 2012 - current.
- Above \$125 effective August 1, 2008 - November 30, 2012.
- Above \$500 effective September 1, 2003 - July 31, 2008.
- Above \$1000 effective August 31, 2003, and prior.

Exception: There is no threshold limit on FAP QC or IPV benefit overpayments.

Exception: There is no threshold limit on CDC provider **system** errors. MiLEAP will recover these types of overpayments.

Example: If payroll checks or electronic funds transfer (EFT) for the CDC provider was issued twice, the full amount would be recovered.

**OVERPAYMENT
PROCESSING****All Programs****ES/FIS Actions**

When a potential overpayment is suspected, the following actions must be taken:

- Immediately correct the **current** benefits if necessary following the change processing requirements in [BAM 220](#).
- **Do not** enter **historical** information relating to the potential overpayment within the Bridges case.
- Complete an Overpayment Referral in Bridges within 60 days. Ensure the following:
 - Select the appropriate overpayment type; agency error (AE) or client error (CE).
 - For all CE overpayments, evaluate the potential overpayment for a suspected IPV; see [BAM 720](#) for IPV criteria. If IPV is suspected, ensure Yes is checked for Suspected IPV.
 - Ensure all relevant evidence is attached to the overpayment referral.

Exception: FAP QC and federal CDC potentially discovered overpayments must be referred within **seven days** of receipt of the FAP QC and CDC audit findings.

FIP, RCA and FAP

Do not refer a potential overpayment if at the time it occurred the group did not include an eligible or disqualified adult.

Medicaid (MA) Only

Do not pursue overpayment of agency errors.

Refer suspected IPV errors to OIG; see [BAM 710](#).

CDC Only

For individuals/clients processed under CDC Expedited Service, [BAM 118](#), benefits issued for child care services provided during the presumptive eligibility period will not be considered an overpayment, regardless of whether the ongoing eligibility determination is approved or denied.

Direct Support Services (DSS), Employment Support Services (ESS) and Family Support Services (FSS)

Initiate a referral to OIG when a suspected IPV results in an overpayment of MDHHS-authorized DSS, including FSS and ESS of \$500 or more; see [BEM 232](#).

State Emergency Relief (SER)

Refer SER overpayments to OIG when IPV overpayment exists and the total overpayment amount exceeds \$500 across all programs. See [Emergency Relief Manual \(ERM\) 306](#) and [ERM 404](#) for SER.

OEA Actions

An OEA will establish an overpayment before the last day of the quarter following the quarter in which the overpayment was discovered. The discovery date may be prior or equal to the establishment date.

Example: If an OEA discovers an overpayment on February 6th (1st quarter), the overpayment claim must be established by June 30th (2nd quarter).

Example: If an OEA discovers an overpayment on June 28th (2nd quarter), the overpayment claim must be established by September 30th (3rd quarter).

Note: An OEA **must** establish overpayment claims even if they cannot be established within the timeframe outlined above per 7 CFR 273.18(d)(3).

For discovering an overpayment, an OEA will:

- Determine if an overpayment actually occurred.
- Determine the overpayment type.
- Determine the overpayment period.
- Obtain all **additional** evidence needed.
- Calculate the amount.
- Establish overpayment(s) for program(s) on BRS.
- Send a DHS-4358-A, -B, -C & -D, Notice of Overissuance, to the client.

FAP QC potentially discovered overpayments must be processed as a **priority** (within 90 days of receipt) because the existence of a potential overpayment has already been verified.

Disaster Relief Program (DRP) and Disaster Food Assistance Program (DFAP) DHS-4701, Overpayment Referrals, must be processed as a priority (within 30 days of receipt); see [BEM 800](#).

OEAs use policy in effect at the time the overpayment occurred.

If after reviewing a client error, it is determined that an IPV is suspected, do not establish a client error in Bridges. Instead, send an overpayment referral to OIG by emailing MDHHS-OIG-IPV-Referral@michigan.gov.

OIG Referral

Prudent judgment should be used in evaluating an overpayment for suspected IPV; see [BAM 720](#) for referral criteria.

IPV is suspected when an overpayment exists for which all three of the following conditions exist:

- The individual/client failed to report information or gave incomplete or inaccurate information needed to make a correct benefit determination, **and**
- The individual/client was clearly and correctly instructed regarding their reporting responsibilities, **and**
- The individual/client has no apparent physical or mental impairment that limits their understanding or ability to fulfill their reporting responsibilities.

Refer all individual/client and CDC provider errors that meet the referral criteria for suspected IPV to OIG. Refer to OIG via an Overpayment Referral in Bridges, ensure Yes is checked for Suspected IPV to route the referral to OIG.

FIP, RCA, and FAP

Do not refer a potential overpayment if at the time it occurred the group did not include an eligible or disqualified adult.

MA, SER and DSS

ES/FIS refer these programs directly to OIG when IPV is suspected. Suspected IPV overpayments are referred to OIG via an Overpayment Referral in Bridges, ensure Yes is checked for Suspected IPV to route the referral to OIG.

- Evidence for the suspected IPV must be attached.

- Provide calculation of the amount and OP period.

These overpayments are not tracked on BRS. OIG will inform the Collections Unit when an accounts receivable needs to be established.

WELFARE FRAUD REFERRAL

The public can contact their local MDHHS office to report suspected welfare fraud or submit a [Welfare Fraud Complaint](#) form online.

Responsibility

Local Office Actions

- Receive a call.
- Enter into online fraud database.
- Complete file clearance.
- Forward to fraud coordinator.

Central Office Actions

- Access and review fraud referrals submitted online by the public via the database.
- Complete file clearance to determine affected county.
- Forward referral electronically to the designated fraud coordinator in local office.

RESOURCES

FIP

Personal Responsibility and Work Opportunity Reconciliation Act of 1996., P. L. 104-193 of 1996, as amended
1939 PA 280 as amended (Social Welfare Act)
MCL 400.1 et seq
MCL 400.60
Mich Admin Code, R 400.3129 - 400.3131

MA

42 CFR 431.230(b)
MCL 400.60

FAP

7 CFR 271.2

7 CFR 272.8

7 CFR 273.18

7 USC 2022

Mich Admin Code, R 400.3011

SDA

Annual Appropriations Act

Mich Admin Code, R 400.3011

RCA

42 CFR 400.49

CDC

The Child Care and Development Block Grant (CCDBG) Act (42 USC § 9858 et seq.), as amended by the CCDBG Act of 2014 (Pub. L. 113-186).

45 CFR Parts 98 and 99.

Social Security Act, as amended 2016.

CONTACT

Policy-Recoupment@michigan.gov - Overpayment Policy

MDHHS-OIG-OEA-Policy@michigan.gov - OIG OEA Policy.